

A STARK ABSENCE
Children having a meal at an Anganwadi centre in Rajkot

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It is an epidemic," the official says. However, a senior public health practitioner, who was earlier roped in for an independent audit of Gujarat's ICDS (Integrated Child Development Services), disagrees with that attempt at rationalisation, stating that the theory is not based on scientific data. "Do we have rich kids being treated for malnutrition?" he asks.

The core issue lies in the lack of effective implementation of ICDS, its honest monitoring, and thus identification of problems. God is in the details, he says. "Do the ASHA and Anganwadi workers regularly visit pregnant and breastfeeding mothers to check on their health? They don't. Do they give the correct information about the nutrition needs to the family? Are they trained and equipped to be able to do this?" he asks.

Research reveals that there is insufficient scientific awareness about what constitutes nutrition, and this unfortunately goes right up to policymaking levels. "The second reason is, well, they cannot afford it," says the public health official. Supplementary food programmes under ICDS lack sufficient micronutrients, and their administration is also not monitored.

The most vulnerable in this situation are girls and women. In rural areas, girls frequently drop out of school, child marriages persist, and young mothers lack both awareness and the support systems that could create it. A startling statistic reveals that 50 per cent of children in Gujarat are not breastfed within the crucial first hour of birth. This grim reality is underpinned by data from NFHS 2015-16, indicating that 24.9 per cent of women aged 20 to 24 were married before reaching the legal age of 18. Notably, the proportion of pregnant women suffering from anaemia in Gujarat rose from 51.3 per cent in 2015-16 to 62.6 per cent in 2019-21.

MALNUTRITION IS THE BIGGEST CONTRIBUTOR TO NITI AAYOG'S MULTIDIMENSIONAL POVERTY INDEX IN GUJARAT

Amid the plethora of schemes from every department, be it that of women and child development, health and family welfare, tribal development or education, the state government is shifting its focus to a comprehensive social awareness campaign. In May, state health minister Rushikesh Patel announced, "We are developing a village-level awareness campaign called 'Maru Gam, Kuposhan Mukt Gam' (My village, malnutrition-free village)."

The health department official outlines their strategy: "We will collaborate with village heads, self-help groups, local school teachers and influential figures from various walks of life to emphasise the importance of maintaining proper nutrition. A well-structured communication plan is being devised."

The success of this campaign hinges on identifying the actual problems in different regions and addressing them on a case-to-case basis. It will call for patience and scientific perseverance. Will Gujarat deliver? ■